

PUSTULAR PSORIASIS

Several clinical variants of pustular psoriasis exist: **generalized pustular psoriasis (von Zumbusch type)**, **annular pustular psoriasis**, **impetigo herpetiformis**, and two variants of localized pustular psoriasis—**pustulosis palmaris et plantaris** and **acrodermatitis continua**. In children, pustular psoriasis can be complicated by sterile, lytic lesions of bones and can be a manifestation of the **SAPHO syndrome** (synovitis, acne, pustulosis, hyperostosis, osteitis).

Generalized Pustular Psoriasis (von Zumbusch)

Generalized pustular psoriasis (von Zumbusch) is a distinctive acute variant of psoriasis. It is usually preceded by other forms of the disease. Attacks are characterized by fever lasting several days and a sudden generalized eruption of sterile pustules 2 to 3 mm in diameter. The pustules are disseminated over the trunk and extremities, including the nail beds, palms, and soles. They typically arise on highly erythematous skin, first as discrete patches and then becoming confluent as the disease progresses. With prolonged disease, the fingertips may become atrophic. The surrounding erythema often spreads and coalesces, potentially leading to erythroderma.

The disease characteristically occurs in waves of fever and pustule formation. The etiology of generalized pustular psoriasis (von Zumbusch type) is unknown. Various triggering factors include infections, irritating topical treatments (via the Koebner phenomenon), and withdrawal of oral corticosteroids. This form is associated with prominent systemic symptoms and may lead to life-threatening complications such as bacterial superinfection, sepsis, and dehydration.

Severe cases can be difficult to control and require potent treatment with rapid onset of action. Commonly used drugs include **etretinate**, **methotrexate (MTX)**, **cyclosporin**, or **oral corticosteroids**. Cases of acute respiratory distress syndrome associated with generalized pustular psoriasis have been reported.

Exanthematic Pustular Psoriasis

Exanthematic pustular psoriasis typically occurs after a viral infection and presents with widespread pustules along with generalized plaque psoriasis. Unlike the von Zumbusch type, it lacks constitutional symptoms and tends not to recur. There is clinical and histopathological overlap between this form and **acute generalized exanthematous pustulosis (AGEP)**, a drug-induced eruption.

Annular Pustular Psoriasis

Annular pustular psoriasis is a rare variant that usually presents in an annular or circinate pattern. Lesions may appear at the onset of pustular psoriasis, spreading to form enlarging rings, or may develop during the course of generalized pustular psoriasis. Characteristic findings include pustules arranged on a ring-shaped erythematous base, sometimes resembling **erythema annulare centrifugum**.

Identical lesions are seen in **impetigo herpetiformis**, which some consider a variant of pustular psoriasis occurring during pregnancy. Onset is typically in the early third trimester and persists until delivery, with earlier onset in subsequent pregnancies. Impetigo herpetiformis is often associated with **hypocalcemia**. Notably, there is usually no personal or family history of psoriasis.

Localized Pustular Psoriasis Variants

Localized forms—**pustulosis palmaris et plantaris** and **acrodermatitis continua of Hallopeau**—are discussed in Chapter 21.

SEBOPSORIASIS

Sebopsoriasis is a common clinical entity characterized by erythematous plaques with greasy scales localized to seborrheic areas, including the scalp, glabella, nasolabial folds, perioral region, presternal area, and intertriginous zones. In the

absence of typical psoriatic lesions elsewhere, differentiation from **seborrheic dermatitis** can be challenging.